A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 April 2026

Dr Amanda Richards
Pinnacle Surgery

Dear Dr Richards,

Re: Mr Lachlan Palamara
DOB: 1994-06-12
Address: 3/58 Maroondah Hwy, CROYDON VIC 3136

Summary

- **Improvement in symptoms of regurgitation and dysphagia following a trial of high-dose acid suppression.**
- **Modest improvement in nocturnal respiratory symptoms while on medication.**
- **Resumption of acid suppression therapy and commencement of low-dose amitriptyline for suspected sensory hypersensitivity.**
- **Review in ten weeks.**
- **No action required by you.**

I had the pleasure of conducting a review with Lachlan to discuss the results of his recent medication trial. As previously planned, Lachlan compared a one-month period of high-dose acid suppression with a subsequent month off therapy to evaluate the impact on his symptoms.

Lachlan reports that his symptoms of regurgitation and dysphagia were notably less frequent while taking the medication. Interestingly, his primary concern regarding his breathing also showed a modest improvement, specifically at nighttime, during the treatment phase. The recurrence of dysphagia upon cessation of the medication suggests that his symptoms are at least partially mediated by gastro-oesophageal reflux. While significant narrowing is not always apparent during routine gastroscopy, chronic reflux can lead to reduced oesophageal elasticity and functional narrowing of the aperture, which appears to be the case here.

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Impression and Plan Given the positive response, I have advised Lachlan to resume his acid suppression therapy. We will maintain this for at least the next six months to manage the reflux-mediated dysphagia, with a view to tapering in the future. Additionally, we discussed the introduction of low-dose amitriptyline, prescribed off-label, to address his respiratory symptoms and suspected sensory hypersensitivity. This medication may also assist by drying mucus secretions, which may be contributing to his discomfort.

I have started Lachlan on 10 mg of amitriptyline at night, with instructions to increase the dose to 20 mg or 30 mg as tolerated. We discussed potential side effects, including sedation, xerostomia, and constipation. I have suggested he take the medication in the evening and initiate the first dose on a weekend to monitor for any significant morning grogginess. I noted his concurrent use of dexamphetamine for ADHD and do not anticipate any significant drug interactions, though we will monitor this closely.

Lachlan understands that it may take four to six weeks to observe a clinical benefit from the amitriptyline. I will review him again in approximately eight to ten weeks to assess his progress.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Paul Tescher

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